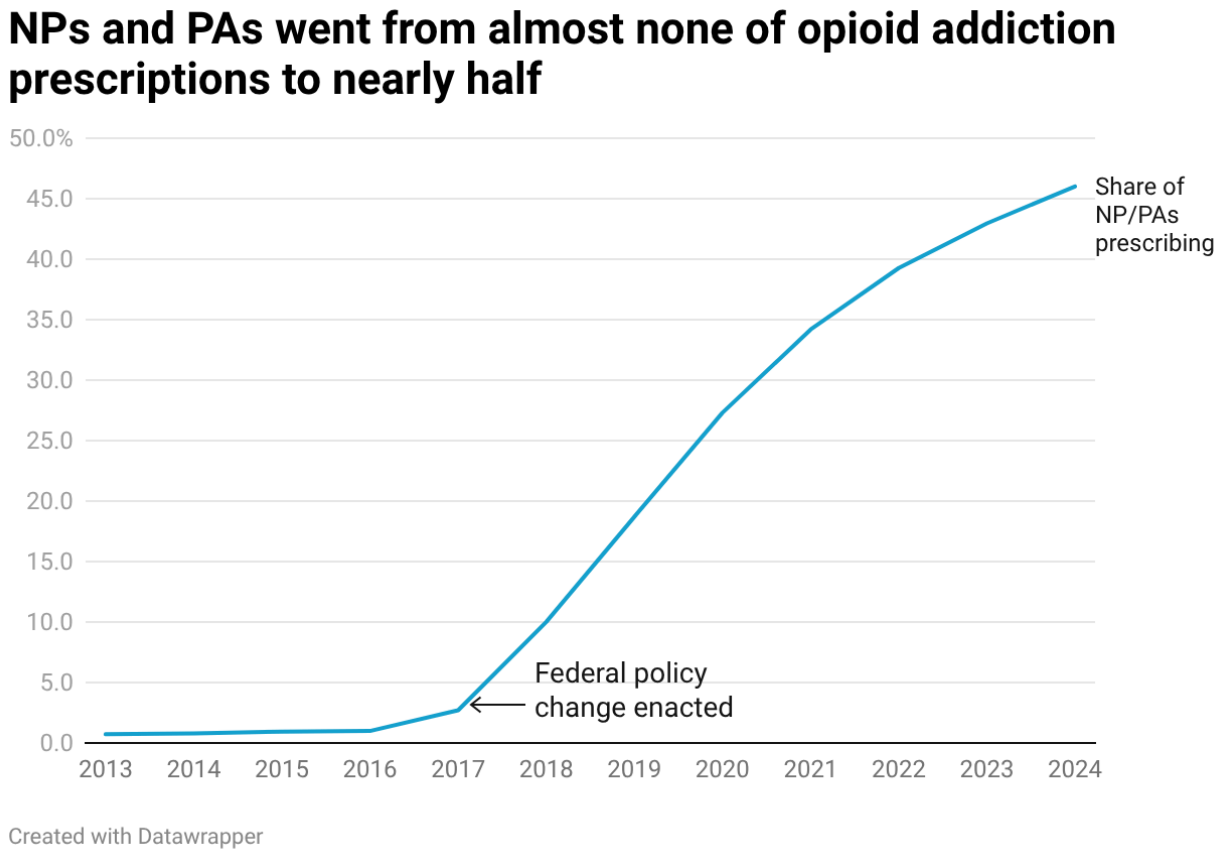
Hed: Nurse practitioners and physician assistants now write nearly half of Medicare's opioid addiction prescriptions.

Dek: Before a 2016 law was passed, NPs and PAs wrote 1% of opioid addiction prescriptions. By 2024, they wrote 46% and accounted for most of the prescription growth.



Link to above chart for tooltips

In 2024, nurse practitioners and physician assistants wrote 46% of Medicare Part D prescriptions for buprenorphine, a medication used to treat opioid addiction. From 2013 – the earliest year in the dataset I used – through 2016, they wrote less than 1%.

They also accounted for most of the growth in treatment. Medicare prescriptions for the addiction medications I tracked (Buprenorphine-Naloxone, Suboxone, Zubsolv, Sublocade, Bunavail, and Brixadi) grew by over 1 million prescriptions from 2016 to 2024. NPs and PAs accounted for 78% of that increase, despite consistently making up the minority of total prescribers.

This matters because buprenorphine helps patients manage opioid addiction, but they can only get it once prescribed. Federal law required a special license to prescribe this medication, but the 2016 Comprehensive Addiction and Recovery Act made NPs and PAs eligible. It became effective in 2017, and knowing who prescribes now shows how much opioid addiction treatment relies on healthcare providers who are not physicians.

This data comes from the Centers for Medicare and Medicaid Services Medicare Part D Prescribes by Provider and Drug Dataset. I "scraped" it from the agency's API using Python, downloading rows from 2013 to 2024 as separate tables. Each table shows information like prescriber type, drug type, number of claims, and state. My universe removed buprenorphine products typically used for pain, so I reduced my table from 394,269 to 289,146 rows. I chose not to remove providers such as students or CRNAs since the number was negligible. In version two, I would need to give greater consideration to prescriber types in order to create the most compelling story. Additionally, I used available Medicare data, so this data excludes Medicaid and private insurance. It also leaves out prescribers with fewer than 11 prescriptions in a year, so this data is imperfect.

I was led to this research as I am interested in health economics, so I wanted to use health data for this project. My sister is in physician assistant school, and we were talking about what medications she can prescribe. I didn't realize that she could prescribe controlled substances, like opioids, which led me to search online databases for prescription requirements and changes across healthcare provider types.

The results partly matched my research question, which hypothesized that policy changes in 2016, 2021, and 2023 would make it easier for NPs and PAs to prescribe opioid treatment medication. Therefore, I thought I would see an increase in prescriptions from NPs and PAs in the years following the three listed above. However, the clearest shift showed that the combined share of prescriptions from NPs and PAs happened after the 2017 policy changed, with little meaningful difference after 2021 or 2023. During the latter two periods, the share of NP and PA prescriptions seemed to grow, but not at an increased rate. This may be because the changes from the enacted policies affected physicians as well, not just NP/PAs.